

Timely Filing Deadline & Claim Submission Control Intelligence™

A student-developed case study that converts a simplified timely-filing infographic into a controlled healthcare operations workflow for payer-rule verification, claim readiness, deadline calculation, submission evidence, rejection recovery, denial prevention, patient financial safeguards, and verified closure.

REVENUE CYCLE

CLAIMS SUPPORT

DENIAL PREVENTION

DATA QUALITY

NO PHI

DOWNLOAD BRANDED PDF

CORE QUESTION

Where did the claim workflow first lose control before the deadline expired?

Created by Kori Pickle, BSHA Candidate, University of Phoenix. This project is educational, simulated, and does not use PHI, employer data, payer portal data, claims data, contracts, or real patient information.

PORTFOLIO PROJECT 22 |
2026-2027

A deadline is not a single date.

It is the final output of a chain of decisions: exact payer and product identification, effective policy retrieval, date-of-service logic, claim type, coordination of benefits, clean-claim readiness, transmission, acceptance evidence, rejection handling, and escalation.

A claim can be submitted "on time" operationally and still fail if it reaches the wrong payer, lacks a clean-claim element, is rejected without follow-up, or has no proof of receipt.

**Kori Pickle | Patient-to-Professional
Healthcare Operations Learner**

What the LinkedIn post explains - and what it leaves out.

The post is useful as an awareness prompt. It should not be used as a universal payer deadline reference.

What it gets right

- Claims must be filed within payer-defined timeframes.
- Late filing can create denials and payment loss.
- Requirements vary by payer and plan.
- Teams should verify payer-specific guidance.

What is operationally missing

- Participation agreement and contract terms
- Original versus corrected versus secondary claim rules
- Date-of-service versus discharge-date logic
- State and product variation
- Proof of submission and clearinghouse acceptance
- Rejected-claim recovery and escalation ownership

Why the table is risky

Brand-wide ranges can look authoritative even when the real deadline depends on the exact product, network status, provider agreement, claim type, effective date, and applicable law. A static social-media chart may become outdated without warning.

Important correction

The infographic states that Medicare has "no filing limit for secondary claims." CMS guidance supports a one-calendar-year Medicare filing rule with limited exceptions; it does not support using a blanket no-limit statement as an operational rule.

Use the infographic to ask better questions - not to calculate a live claim deadline.

Career relevance: This distinction demonstrates evidence validation, policy-source discipline, revenue-cycle awareness, and the ability to translate public content into a safer operational control.

The exact source matters more than the payer logo.

Examples below are educational and current to the cited 2026 sources. Always validate the live policy and contract before operational use.

Program / payer example	What the authoritative source says	Operational implication	Control lesson
Original Medicare	Submit to the correct Medicare Administrative Contractor no later than one calendar year after the date of service, subject to limited exceptions.	A general "12 months" label is incomplete unless the team confirms the correct contractor, date logic, and exception documentation.	Store the source, effective date, deadline basis, and exception rationale.
TRICARE	Claims in the United States and U.S. territories generally must be filed within one year of service; overseas claims generally have three years.	Location changes the filing window. A single 365-day statement does not cover every TRICARE scenario.	Capture service location and beneficiary context before calculating the deadline.
Cigna Healthcare	The public provider page lists 90 days for participating providers and 180 days for out-of-network providers, with stated exceptions.	Network participation and coordination-of-benefits circumstances may change the rule.	Exact network status and exception evidence are required fields.
UnitedHealthcare	The 2026 administrative guide directs providers to their internal contracting contact or Participation Agreement for timely-filing information.	A generic logo-based deadline may conflict with the controlling agreement.	Contract source hierarchy must be built into the workflow.
TennCare / MCO workflows	TennCare directs providers to applicable managed-care organization processes and has announced a future filing-limit change for certain claim categories effective July 1, 2027.	State Medicaid rules, MCO manuals, claim categories, and effective dates can change.	Policy monitoring and effective-date version control are essential.

Rule source Contract, provider manual, government manual, portal notice, or controlling law.	Rule context Plan, product, network, claim type, date logic, state, and effective date.	Rule evidence Source title, version, retrieval date, screenshot or document reference, and reviewer.
--	---	--

The denial appears at the end. Control is often lost much earlier.

01 Service completed	02 Charge captured	03 Documentation complete	04 Payer identified	05 Rule verified	06 Claim created	07 Claim submitted	08 Receipt accepted	09 Rejection resolved	10 Closure verified
--------------------------------	------------------------------	-------------------------------------	-------------------------------	----------------------------	----------------------------	------------------------------	-------------------------------	---------------------------------	-------------------------------

Failure point	Visible downstream problem	Why the deadline control failed	Patient / operational consequence
Incorrect product selected	Claim routed to wrong payer or address	The workflow used a payer brand instead of the exact plan and contract.	Delayed adjudication, avoidable rework, possible patient bill.
Documentation incomplete	Claim held in work queue	No aging threshold or escalation owner existed.	Deadline days consumed before the claim was even created.
Static deadline table	Incorrect filing date entered	Source version and effective date were not controlled.	False sense of readiness and missed filing window.
Transmission only	Rejected claim assumed filed	"Sent" was treated as "accepted" without acknowledgement review.	Claim never entered payer adjudication.
No proof of original filing	Late-filing dispute cannot be supported	Submission evidence was not stored with the claim record.	Contractual write-off or lost reimbursement exposure.
COB hold without reset logic	Secondary claim ages silently	Primary EOB date and secondary filing rule were not linked.	Patient confusion and delayed balance resolution.

FIRST LOSS OF CONTROL

The organization treated timely filing as a static payer lookup instead of a source-controlled, evidence-based, closed-loop workflow.

A 24-claim queue with hidden deadline exposure.

All values are synthetic and created for educational demonstration only.

A simulated specialty practice reviews a mixed-payer claim queue. Staff have a spreadsheet containing payer names and broad filing ranges, but the file does not record the exact product, contract source, effective date, claim type, acknowledgement status, or proof of original submission.

24 SYNTHETIC CLAIMS	7 WITHIN 30 DAYS	5 MISSING RULE SOURCE	4 NO RECEIPT EVIDENCE
------------------------	---------------------	--------------------------	--------------------------

ID	Payer context	Claim status	Days remaining	Control gap	Priority
TF-003	Commercial participating	Documentation hold	11	No owner; rule copied from old spreadsheet	Critical
TF-006	Original Medicare	EDI rejected	17	Rejection not routed to correction queue	Critical
TF-009	TRICARE U.S.	Ready to submit	22	No stored service-location evidence	High
TF-012	Commercial secondary	Waiting for primary EOB	Unknown	Secondary rule and trigger date not documented	High
TF-015	Medicaid MCO	Corrected claim	14	Original claim receipt not linked	Critical
TF-018	Commercial out-of-network	Submitted	28	Clearinghouse acceptance not confirmed	High
TF-021	Medicare Advantage	Provider edit hold	19	Deadline assumed from Original Medicare	Critical
TF-024	Commercial participating	Pending attachment	26	No internal safety deadline	High

Baseline finding

The most dangerous records were not necessarily the oldest. They were the claims with uncertain rule sources, silent rejections, unresolved ownership, or missing proof.

Patient-to-professional insight

A missed filing deadline can become a patient experience problem when balances are delayed, corrected late, transferred incorrectly, or communicated without clear explanation.

Timely Filing Closed-Loop Control Model™

01

IDENTIFY
exact payer product

02

VERIFY
authoritative rule

03

CALCULATE
external + safety dates

04

PREPARE
clean claim evidence

05

SUBMIT
approved channel

06

CAPTURE
receipt evidence

07

MONITOR
acknowledgement

08

CORRECT
rejection or defect

09

ESCALATE
aging exception

10

CLOSE
adjudication verified

Control field	Required content	Why it matters	Escalation trigger
Exact coverage context	Payer, product, plan, network status, state, line of business	Prevents brand-level assumptions.	Coverage context incomplete or conflicting.
Claim type	Original, corrected, replacement, void, secondary, appeal, reconsideration	Different pathways may have different timeframes.	Claim type unknown or changed after submission.
Deadline basis	Date of service, discharge, primary adjudication, notice, or contract-defined event	Ensures the correct start date is used.	Trigger date is unavailable or disputed.
Authoritative source	Contract/manual title, section, version, effective date, reviewer	Supports traceability and policy validation.	No current source or source conflict.
Internal safety date	External deadline minus approved buffer	Creates time for correction and resubmission.	Claim crosses high-risk aging threshold.
Submission evidence	Transaction control number, clearinghouse report, portal receipt, timestamp	Distinguishes "sent" from "accepted."	No acknowledgement within defined interval.
Closure evidence	Accepted, adjudicated, corrected, appealed, or authorized exception	Prevents silent work-queue abandonment.	Claim remains unresolved after escalation.

A safer alternative to a static payer cheat sheet.

The matrix stores evidence and logic instead of relying on memory or a social-media table.

Field	Example value	Validation requirement	Status vocabulary
Coverage context	Cigna participating commercial plan	Verify ID card, eligibility result, contract/network context	Confirmed / Conflict / Unresolved
Rule source	Cigna provider claims page	Record retrieval date and applicable exception	Current / Superseded / Unknown
External deadline	Calculated from authoritative rule	Formula review by second person or automated validation	Verified / Review needed
Internal safety date	30 days before external deadline	Organization-defined policy, not payer rule	Safe / Warning / Critical
Submission channel	837 EDI via approved clearinghouse	Correct payer ID and transaction format	Ready / Blocked / Submitted
Receipt evidence	999/277CA or clearinghouse acceptance report	Confirm acceptance, not only transmission	Accepted / Rejected / Pending
Correction pathway	Corrected claim linked to original control number	Follow exact payer and contract requirements	Open / Resubmitted / Closed
COB evidence	Primary EOB and primary processing date	Confirm secondary rule trigger and required attachment	Waiting / Ready / Submitted
Exception evidence	Retroactive eligibility notice or payer-directed resubmission	Store documentation and approval pathway	Potential / Validated / Denied
Closure	Adjudicated or documented final resolution	Patient balance safeguards and denial feedback completed	Closed / Follow-up / Escalated

SAFE

More than 45 days before internal safety date; source and evidence complete.

WARNING

15-45 days remaining or one unresolved control field.

CRITICAL

Fewer than 15 days, rejected, missing source, or no accountable owner.

EXCEPTION

Deadline passed or rule disputed; formal review and evidence pathway required.

The deadline field should never be editable without the source, trigger date, claim type, and reviewer changing with it.

Test the workflow before trusting it.

Test	Scenario	Expected control behavior	Pass evidence
UAT-01	Payer logo matches multiple products with different contracts.	System blocks deadline calculation until exact product and network context are confirmed.	Blocked status, reason code, owner, timestamp.
UAT-02	Claim is transmitted but receives a clearinghouse rejection.	Status changes to rejected; deadline clock remains active; correction task routes to named owner.	Rejection code, correction due date, resubmission receipt.
UAT-03	Original Medicare claim is 20 days from external deadline.	Critical alert activates and requires daily follow-up until acceptance or documented exception.	Alert history and closure evidence.
UAT-04	Secondary claim is waiting for primary EOB.	Record stores primary processing date, secondary rule source, attachment requirement, and escalation date.	EOB link and verified secondary deadline.
UAT-05	Corrected claim is created without the original claim reference.	Submission is blocked until original control number and payer-specific corrected-claim rule are documented.	Validation message and completed reference field.
UAT-06	Provider manual is replaced with a new effective version.	Open claims are re-evaluated when the effective-date logic applies; superseded source remains auditable.	Version history and impacted-claim report.
UAT-07	No acknowledgement is received after the expected interval.	System creates a follow-up task instead of assuming successful filing.	Follow-up record and payer/clearinghouse response.
UAT-08	Deadline passes while documentation remains incomplete.	Claim enters formal exception review; patient balance is protected from automatic transfer until review is complete.	Exception decision and patient-account hold evidence.

What this demonstrates

Business requirements, status design, exception routing, data validation, policy version control, user acceptance testing, and patient-impact safeguards.

What this does not claim

No payer portal use, billing authority, coding authority, contract interpretation authority, claims submission experience, or employer outcomes.

KPIs should expose control risk before a denial occurs.

Metrics are designed for a simulated dashboard and should be adapted to organizational policy.

Deadline source completeness Percent of open claims with current rule source, version, effective date, and reviewer.	First-pass acceptance rate Percent of submitted claims accepted by the clearinghouse or payer without rejection.	Critical aging rate Percent of claims inside the critical threshold before the internal safety date.
Unacknowledged submission rate Percent of transmitted claims without receipt or acceptance evidence.	Timely-filing denial rate Percent of adjudicated claims denied for filing-limit reasons, segmented by failure origin.	Exception recovery rate Percent of validated exceptions resolved with documented evidence and final disposition.

Dashboard view	Question answered	Recommended segmentation	Action
Deadline risk heatmap	Which claims may miss the internal or external deadline?	Payer product, claim type, owner, days remaining	Prioritize critical claims and assign next action.
Failure-origin trend	Where does timely-filing exposure begin?	Documentation, charge capture, payer ID, rejection, COB, ownership	Correct the upstream control, not only the final denial.
Submission evidence gap	Which claims cannot prove acceptance?	Channel, clearinghouse, payer, owner	Retrieve evidence or escalate before deadline.
Policy-version impact	Which claims rely on a superseded rule?	Rule source, effective date, product	Recalculate and document the updated requirement.
Patient financial safeguard queue	Which patient balances require hold or review?	Denial reason, filing status, exception status	Prevent premature or unsupported patient billing.

EXECUTIVE INSIGHT

A low timely-filing denial rate can still hide serious risk if staff are manually rescuing claims at the last moment. Measure both outcomes and control reliability.

AI may support the workflow, but it cannot become the rule source.

Human review remains required when deadlines, contracts, exceptions, or patient balances are affected.

Appropriate support

- Extract candidate dates from an approved source.
- Flag missing fields or conflicting plan information.
- Draft a non-clinical work-queue summary.
- Identify aging patterns in synthetic data.
- Suggest a follow-up checklist for human review.

Inappropriate or high-risk use

- Invent a payer deadline from general knowledge.
- Interpret a contract as final authority.
- Approve a late-filing exception.
- Close a rejected claim without evidence.
- Transfer a balance to the patient automatically.

AI control	Required safeguard	Failure signal	Human owner
Approved-source restriction	Use only designated current manuals, contracts, and official notices.	Source missing, external web content, or superseded version.	Revenue-cycle policy owner or authorized reviewer.
Draft status	AI-calculated deadline remains draft until validated.	Deadline becomes active without reviewer identity.	Assigned claims-support reviewer.
Traceability	Store source text, date logic, model output, and reviewer action.	Deadline exists without reproducible calculation.	Data-quality or workflow owner.
Uncertainty handling	Route conflicting or incomplete records to exception review.	AI selects a rule despite product ambiguity.	Contracting / payer-relations escalation pathway.
Patient safeguard	Do not use unresolved filing status to assign patient responsibility.	Balance changes while claim status is disputed.	Patient financial-services reviewer.

Responsible automation makes uncertainty more visible. It does not hide uncertainty behind a confident date.

How this project supports Kori's 2026-2027 career foundation.

Revenue-cycle support

Demonstrates claim-lifecycle awareness, deadline risk, submission evidence, rejection recovery, and denial feedback.

Patient access

Connects accurate coverage context and patient communication to downstream claim readiness and financial clarity.

Health informatics support

Shows data-field design, status vocabulary, source traceability, version control, and workflow integration.

Quality improvement

Uses root-cause analysis, measurable controls, UAT, exception tracking, and closed-loop verification.

Implementation support

Translates policy into business requirements, ownership rules, alerts, testing scenarios, and adoption safeguards.

Responsible AI

Defines approved support, prohibited authority, human review, traceability, and patient-impact boundaries.

Resume project bullet:

Developed a student-level, no-PHI timely-filing control case study that translated payer-rule variability into an evidence-based claim workflow; designed source-version fields, deadline calculations, internal safety thresholds, submission-receipt controls, rejection escalation, synthetic UAT scenarios, KPIs, patient financial safeguards, and responsible-AI boundaries.

Interview talking point:

I learned that timely filing is not just a deadline lookup. It is a closed-loop operational control. The team must identify the exact payer product, verify the controlling source, calculate the correct trigger date, submit a clean claim, capture acceptance evidence, resolve rejections, and confirm closure. My simulated project shows how I would organize those requirements without claiming billing or payer-portal experience.

CAREER INSIGHT TO SAVE

Recruiter-ready value comes from showing how a policy becomes a reliable workflow - with data, ownership, evidence, escalation, testing, and patient safeguards.

Evidence-based, current, and deliberately bounded.

Accessed July 2026. Rules may change. The controlling contract, manual, law, or payer notice must be verified for each live workflow.

1. Centers for Medicare & Medicaid Services. *Timely Filing - Medicare Billing: CMS-1500 & 837P*. CMS states that Medicare claims must be submitted to the correct MAC no later than one calendar year after the date of service. <https://www.cms.gov/Outreach-and-Education/MLN/WBT/MLN4462429-MLN-WBT-1500/1500/lesson01/08/index.html>
2. Centers for Medicare & Medicaid Services. *Exceptions for Late Filing - Medicare Billing: CMS-1500 & 837P*. CMS describes limited exceptions, including administrative error, retroactive entitlement, certain Medicaid recoupment situations, and retroactive disenrollment from MA/PACE. <https://www.cms.gov/outreach-and-education/mln/wbt/mln4462429-mln-wbt-1500/1500/lesson01/09/index.html>
3. TRICARE. *Filing Claims*. TRICARE states that U.S. and U.S.-territory claims generally must be filed within one year of service and other overseas claims within three years. <https://www.tricare.mil/PatientResources/Claims>
4. Cigna Healthcare. *Claims Submission, Payments, and Filing*. Cigna's public provider page lists 90 days for participating providers and 180 days for out-of-network providers, with exceptions. <https://www.cigna.com/health-care-providers/coverage-and-claims/submit-claims>
5. UnitedHealthcare. *2026 UnitedHealthcare Care Provider Administrative Guide*. The guide directs providers to their internal contracting contact or Participation Agreement for timely-filing information and notes that the guide is subject to change. <https://www.uhcprovider.com/content/dam/provider/docs/public/admin-guides/2026-UHC-Administrative-Guide.pdf>
6. Tennessee Division of TennCare. *TennCare Provider Resource Board*. TennCare provides provider updates and announced a future timely-filing change for certain 1915(c) and ICF/IID claims effective July 1, 2027. <https://www.tn.gov/tenncare/providers/tenncare-provider-resource-board.html>

Educational limitation

This project does not provide legal, contractual, coding, billing, or payer-specific operational advice. It demonstrates a student-designed control model.

Integrity boundary

Kori Pickle does not claim claims-submission experience, payer-portal access, contract interpretation authority, coding authority, billing authority, or employer results.

HEALTHCARE OPERATIONS INTELLIGENCE ENGINE™

Where healthcare workflows break before patients, staff, and revenue feel the impact.

Created by Kori Pickle, BSHA Candidate, University of Phoenix. Student-developed, simulated, no-PHI healthcare operations portfolio project.